Flohale 125 mcg
Inhaler CFC free

21096957

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Patient Information Leaflet

Fluticasone Propionate Inhaler

flohale inhaler

Before using your **flohale** inhaler read this leaflet carefully and follow the instructions.

CFC-FREE

Cipla

Parts Of The Inhaler

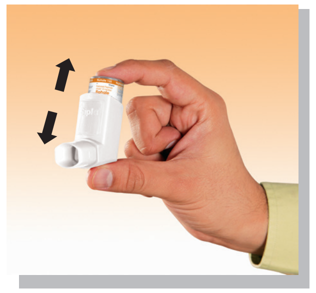
How To Use Your Inhaler Correctly

Testing your Inhaler

Before using your Inhaler for the first time, or if it has not been used for a week or more, "**test fire**" it, i.e. release one puff into the air.

Using your inhaler

1. Remove the mouthpiece cover, and check that the mouthpiece is clean. Hold the inhaler upright as shown, with your thumb on the base. Place either one or two fingers on top of the canister.



2. Breathe out gently, through your mouth.

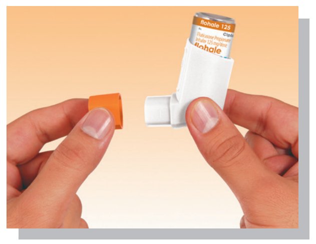
3. Place the mouthpiece of the inhaler in your mouth between your teeth and close your lips around it (Do not bite it).

Start breathing in slowly through your mouth. As you breathe in, press down the canister to release one dose while continuing to breathe in steadily and deeply.

4. Remove the inhaler from your mouth and hold your breath for 10 seconds, or for as long as it is comfortable. Breathe out slowly.

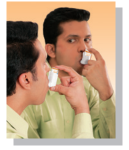
Rinse the mouth after each use

5. If another dose is required wait for at least one minute. Repeat steps 2 to 4. After use, replace the mouthpiece cover.



Important:

Do not rush steps 3 and 4.
It is important that you start to breathe in as slowly as



possible just before releasing the dose. Practise in front of the mirror for the first few times.

If you see "mist" coming from the top of the inhaler or the sides of your mouth, start again from step 1. This escaping mist indicates incorrect technique.

Cleansing

It is essential to keep the plastic mouthpiece clean, to ensure proper functioning of the inhaler. Clean your inhaler at least once a week, as follows:

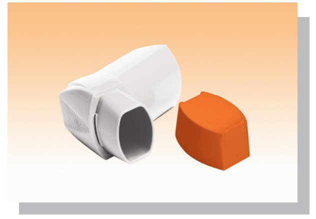
1. Gently pull the metal canister out of the plastic body of the inhaler. Remove the mouthpiece cover.

2. Immerse the plastic body and the mouthpiece cover in warm water. **Do not put the metal canister in the water.**

3. Next, rinse the plastic body and mouthpiece cover under running tap water.

4. Shake well to remove excess water.

Leave to dry. Avoid the use of heat.



5. When the plastic body is dry, replace the canister and the mouthpiece cover correctly.

Warning

If the recommended dose does not provide the desired effect, consult your doctor. It is dangerous to exceed the recommended dose.

The metal canister is pressurised. Do not puncture or burn it even when empty.

Keep away from eyes.

Keep away from children.

21096957

For the use of Registered Medical Practitioner or a Hospital or a Laboratory only

Fluticasone Propionate Inhaler

Flohale 125 mcg Inhaler CFC free

COMPOSITION
Each actuation delivers
Fluticasone propionate BP........125 mcg
Suspended in Propellant-134a .... q.s.
Ethanol .... 2% w/w

DOSAGE FORM
Pressurised Inhalation

Description
Flohale 125 Inhaler is a white homogeneous suspension aerosol for inhalation, in propellant HFA -134a, supplied in a pressurised container. On visual examination there should be no sign of physical damage, or leakage

Pharmacology
Fluticasone propionate given by inhalation at recommended doses has a potent glucocorticoid anti-inflammatory action within the lungs, resulting in a reduction of both symptoms and exacerbations of asthma, with a lower incidence and severity of adverse effects than those observed when corticosteroids are administered systemically.

Pharmacokinetics
In healthy subjects the mean systemic bioavailability of fluticasone HFA inhaler is 28.6%. In patients with asthma (FEV_1 < 75% predicted) the mean systemic absolute bioavailability was reduced by 62%. Systemic absorption occurs mainly through the lungs and has been shown to be linearly related to dose over the dose range 500 to 2000 micrograms. Absorption is initially rapid then prolonged and the remainder of the dose may be swallowed.

Absolute oral bioavailability is negligible (<1%) due to a combination of incomplete absorption from the GI tract and extensive first-pass metabolism.

87-100% of an oral dose is excreted in the faeces, up to 75% as parent compound. There is also a non-active major metabolite.

After an intravenous dose, fluticasone propionate is extensively distributed in the body. The very high clearance rate indicates extensive hepatic clearance.

Indications
Asthma
Fluticasone propionate has a marked anti-inflammatory effect in the lungs. It reduces symptoms and exacerbations of asthma in patients previously treated with bronchodilator alone or with other prophylactic therapy. Severe asthma requires regular medical assessment as death may occur. Patients with severe asthma have constant symptoms and frequent exacerbations, with limited physical capacity, and PEF values below 60% predicted at baseline with greater than 30% variability, usually not returning entirely to normal after a bronchodilator. These patients will require high dose inhaled (see dosage instructions) or oral corticosteroid therapy. Sudden worsening of symptoms may require increased corticosteroid dosage which should be administered under urgent medical supervision.

Adults Prophylactic management in:-
Mild asthma (PEF values greater than 80% predicted at baseline with less than 20% variability): Patients requiring intermittent symptomatic bronchodilator asthma medication on more than an occasional basis.
Moderate asthma (PEF values 60-80% predicted at baseline with 20-30% variability): Patients requiring regular asthma medication and patients with unstable or worsening asthma on currently available prophylactic therapy or bronchodilator alone.
Severe asthma (PEF values less than 60% predicted at baseline with greater than 30% variability): Patients with severe chronic asthma.

On introduction of inhaled fluticasone propionate many patients who are dependent on systemic corticosteroids for adequate control of symptoms may be able to reduce significantly or to eliminate their requirement for oral corticosteroids.

Children:
Any child who requires prophylactic medication, including patients not controlled on currently available prophylactic medication.

Dosage & Administration
Flohale CFC free inhaler is for oral inhalation use only. Flohale CFC free inhaler may be used with a spacer device by patients who find it difficult to synchronize aerosol actuation with inspiration of breath.

Patients should be made aware of the prophylactic nature of therapy with Flohale CFC free and that it should be taken regularly even when they are asymptomatic. The onset of therapeutic effect is 4 to 7 days, although some benefit may be apparent as soon as 24 hours for patients who have not previously received inhaled steroids.

Adults and children over 16 years:
100 to 1,000 micrograms twice daily, usually as two twice daily inhalations.

Prescribers should be aware that fluticasone propionate is as effective as other inhaled steroids approximately at half the microgram daily dose. For example, a 100mcg of fluticasone propionate is approximately equivalent to 200mcg dose of beclometasone dipropionate (CFC containing) or budesonide.

Patients should be given a starting dose of inhaled fluticasone propionate which is appropriate to the severity of their disease.

Typical Adult Starting Doses:
For patients with mild asthma, a typical starting dose is 100 micrograms twice daily. In moderate and more severe asthma, starting doses may need to be 250 to 500 micrograms twice daily. Where additional clinical benefit is expected, doses of up to 1000 micrograms twice daily may be used. Initiation of such doses should be prescribed only by a specialist in the management of asthma (such as a consultant physician or general practitioner with appropriate experience).

The dose should be titrated down to the lowest dose at which effective control of asthma is maintained

Children 4 years of age and over:
50 to 200 mg twice daily.

Many children's asthma will be well controlled using the 50 to 100 microgram twice daily dosing regime. For those patients whose asthma is not sufficiently controlled, additional benefit may be obtained by increasing the dose up to 200 micrograms twice daily. The maximum licensed dose in children is 200 micrograms twice daily.

The starting dose should be appropriate to the severity of the disease. The dose should be titrated down to the lowest dose at which effective control of asthma is maintained.

This presentation of fluticasone propionate may not offer the required paediatric dose, in which case an alternative presentation of fluticasone propionate should be considered (e.g. dry powder inhalers). It should not be used for children under 4 years.

Special patient groups:
There is no need to adjust the dose in elderly patients or those with hepatic or renal impairment.

Contraindications
Hypersensitivity to any ingredient of the preparation.

Warning & Precautions
Patient's inhaler technique should be checked regularly to make sure that inhaler actuation is synchronized with inspiration to ensure optimum delivery to the lungs. During inhalation, the patient should preferably sit or stand. The inhaler has been designed for use in a vertical position.

Flohale 125 inhaler CFC Free is not designed to relieve acute symptoms for which an inhaled short-acting bronchodilator is required. Patients should be advised to have such rescue medication available.
Severe asthma requires regular medical assessment, including lung-function testing, as patients are at risk of severe attacks and even death. Increasing use of short-acting inhaled β_2-agonists to relieve symptoms indicates deterioration ofasthma control. If patients find that short-acting relief bronchodilator treatment becomes less effective, or they need more inhalations than usual, medical attention must be sought. In this situation patients should be reassessed and consideration given to the need for increased anti-inflammatory therapy (e.g. higher doses of inhaled corticosteroids or a course of oral corticosteroids). Severe exacerbations of asthma must be treated in the normal way.
There have been very rare reports of increases in blood glucose levels, in patients with or without a history of diabetes mellitus. This should be considered in particular when prescribing to patients with a history of diabetes mellitus.
As with other inhalation therapy, paradoxical bronchospasm may occur with an immediate increase in wheezing after dosing. Flohale 125 inhaler CFC Free should be discontinued immediately, the patient assessed and alternative therapy instituted if necessary.

Systemic effects of inhaled corticosteroids may occur, particularly at high doses prescribed for prolonged periods. These effects are much less likely to occur than with oral corticosteroids. Possible systemic effects include Cushing's syndrome, Cushingoid features, and adrenal suppression, growth retardation in children and adolescents, decrease in bone mineral density, cataract and glaucoma. It is important therefore that the dose of inhaled corticosteroid is reviewed regularly and reduced to the lowest dose at which effective control of asthma is maintained.

Prolonged treatment with high doses of inhaled corticosteroids may result in adrenal suppression and acute adrenal crisis. Children aged < 16 years taking higher than licensed doses of fluticasone (typically 1000mcg/day) may be at particular risk. Situations, which could potentially trigger acute adrenal crisis, include trauma, surgery, infection or any rapid reduction in dosage. Presenting symptoms are typically vague and may include anorexia, abdominal pain, weight loss, tiredness, headache, nausea, and vomiting, decreased level of consciousness, hypoglycaemia, and seizures. Additional systemic corticosteroid cover should be considered during periods of stress or elective surgery.

It is recommended that the height of children receiving prolonged treatment with inhaled corticosteroids is regularly monitored. If growth is slowed, therapy should be reviewed with the aim of reducing the dose of inhaled corticosteroid, if possible, to the lowest dose at which effective control of asthma is maintained. In addition, consideration should be given to referring the patient to a paediatric respiratory specialist.
Administration of high doses, above 1000 mcg daily is recommended through a spacer to reduce side effects in the mouth and throat. However, as systemic absorption is largely through the lungs, the use of a spacer plus metered dose inhaler may increase drug delivery to the lungs. It should be noted that this could potentially lead to an increase in the risk of systemic adverse effects. A lower dose may be required.

The benefits of inhaled fluticasone propionate should minimise the need for oral steroids. However, patients transferred from oral steroids, remain at risk of impaired adrenal reserve for a considerable time after transferring to inhaled fluticasone propionate. The possibility of adverse effects may persist for some time. These patients may require specialised advice to determine the extent of adrenal impairment before elective procedures. The possibility of residual impaired adrenal response should always be considered in emergency (medical or surgical) and elective situations likely to produce stress, and appropriate corticosteroid treatment considered.

Lack of response or severe exacerbations of asthma should be treated by increasing the dose of inhaled fluticasone propionate and, if necessary, by giving a systemic steroid and/or an antibiotic if there is an infection.
Replacement of systemic steroid treatment with inhaled therapy sometimes unmasks allergies such as allergic rhinitis or eczema previously controlled by the systemic drug. These allergies should be symptomatically treated with antihistamine and/or topical preparations, including topical steroids.

As with all inhaled corticosteroids, special care is necessary in patients with active or quiescent pulmonary tuberculosis.

Treatment with Flohale 125 inhaler CFC Free should not be stopped abruptly.

For the transfer of patients being treated with oral corticosteroids:
The transfer of oral steroid-dependent patients to Flohale 125 inhaler CFC Free and their subsequent management needs special care as recovery from impaired adrenocortical function, caused by prolonged systemic steroid therapy, may take a considerable time.

Patients who have been treated with systemic steroids for long periods of time or at a high dose may have adrenocortical suppression. With these patients adrenocortical function should be monitored regularly and their dose of systemic steroid reduced cautiously.

After approximately a week, gradual withdrawal of the systemic steroid is commenced. Decrements in dosages should be appropriate to the level of maintenance systemic steroid, and introduced at not less than weekly intervals. For maintenance doses of prednisolone (or equivalent) of 10mg daily or less, the decrements in dose should not be greater than 1mg per day, at not less than weekly intervals. For maintenance doses of prednisolone in excess of 10mg daily, it may be appropriate to employ cautiously, larger decrements in dose at weekly intervals.

Some patients feel unwell in a non-specific way during the withdrawal phase despite maintenance or even improvement of the respiratory function. They should be encouraged to persevere with inhaled fluticasone propionate and to continue withdrawal of systemic steroid, unless there are objective signs of adrenal insufficiency.
Patients weaned off oral steroids whose adrenocortical function is still impaired should carry a steroid warning card indicating that they need supplementary systemic steroid during periods of stress, e.g. worsening asthma attacks, chest infections, major intercurrent illness, surgery, trauma, etc.

Ritonavir can greatly increase the concentration of fluticasone propionate in plasma. Therefore, concomitant use should be avoided, unless the potential benefit to the patient outweighs the risk of systemic corticosteroid side-effects. There is also an increased risk of systemic side effects when combining fluticasone propionate with other potent CYP3A inhibitors.

Drug Interactions
Under normal circumstances, low plasma concentrations of fluticasone propionate are achieved after inhaled dosing, due to extensive first pass metabolism and high systemic clearance mediated by cytochrome P450 3A4 in the gut and liver. Hence, clinically significant drug interactions mediated by fluticasone propionate are unlikely.

In an interaction study in healthy subjects with intranasal fluticasone propionate, ritonavir (a highly potent cytochrome P450 3A4 inhibitor) 100 mg b.i.d. increased the fluticasone propionate plasma concentrations several hundred fold, resulting in markedly reduced serum cortisol concentrations. Information about this interaction is lacking for inhaled fluticasone propionate, but a marked increase in fluticasone propionate plasma levels is expected. Cases of Cushing's syndrome and adrenal suppression have been reported. The combination should be avoided unless the benefit outweighs the increased risk of systemic glucocorticoid side-effects.

In a small study in healthy volunteers, the slightly less potent CYP3A inhibitor ketoconazole increased the exposure of fluticasone propionate after a single inhalation by 150%. This resulted in a greater reduction of plasma cortisol as compared with fluticasone propionate alone. Co-treatment with other potent CYP3A inhibitors, such as itraconazole, is also expected to increase the systemic fluticasone propionate exposure and the risk of systemic side-effects. Caution is recommended and long-term treatment with such drugs should, if possible, be avoided.

Pregnancy & Lactation
There is inadequate evidence of safety of fluticasone propionate in human pregnancy. Data on a limited number (200) of exposed pregnancies indicate no adverse effects of Flohale 125 Inhaler CFC Free on pregnancy or the health of the foetus/new born child. To date no other relevant epidemological data are available. Administration of corticosteroids to pregnant animals can cause abnormalities of fetal development, including cleft palate and intra-uterine growth retardation. There may therefore be a very small risk of such effects in the human fetus. It should be noted, however, that the fetal changes in animals occur after relatively high systemic exposure. Because Flohale 125 Inhaler CFC Free delivers fluticasone propionate directly to the lungs by the inhaled route it avoids the high level of exposure that occurs when corticosteroids are given by systemic routes. Administration of fluticasone propionate during pregnancy should only be considered if the expected benefit to the mother is greater than any possible risk to the fetus.
The secretion of fluticasone propionate in human breast milk has not been investigated. Subcutaneous administration of fluticasone propionate to lactating laboratory rats produced measurable plasma levels and evidence of fluticasone propionate in the milk. However, plasma levels in humans after inhalation at recommended doses are likely to be low. When fluticasone propionate is used in breast-feeding mothers the therapeutic benefits must be weighed against the potential hazards to mother and baby.

Undesirable effects
Adverse events are listed below by system organ class and frequency. Frequencies are defined as: very common (≥1/10), common (≥1/100 and <1/10), uncommon (≥1/1000 and <1/100), rare (≥1/10,000 and <1/1000) and very rare (<1/10,000) including isolated reports. Very common, common and uncommon events were generally determined from clinical trial data. Rare and very rare events were generally determined from spontaneous data.

System Organ Class	Adverse Event	Frequency
Infections & Infestations	Candidiasis of the mouth and throat	Very Common
Immune System Disorders	Hypersensitivity reactions with the following manifestations:	
	Cutaneous hypersensitivity reactions	Uncommon
	Angioedema (mainly facial and oropharyngeal oedema)	Very Rare
	Respiratory symptoms (dyspnoea and/or bronchospasm)	Very Rare
	Anaphylactic reactions	Very Rare
Endocrine Disorders	Cushing's syndrome, Cushingoid features, adrenal suppression, growth retardation in children and adolescents, decreased bone mineral density, cataract, glaucoma	Very Rare
Metabolism & Nutrition Disorders	Hyperglycaemia	Very Rare
Gastrointestinal Disorders	Dyspepsia	Very Rare
Musculoskeletal & Connective Tissue Disorders	Arthralgia	Very Rare
Psychiatric Disorders	Anxiety, sleep disorders, behavioural changes, including hyperactivity and irritability (predominantly in children)	Very Rare
Respiratory, Thoracic & Mediastinal Disorders	Hoarseness/dysphonia	Common
	Paradoxical bronchospasm	Very Rare

Hoarseness and candidiasis of the mouth and throat (thrush) occurs in some patients. Such patients may find it helpful to rinse out their mouth with water after using the inhaler. Symptomatic candidiasis can be treated with topical anti-fungal therapy whilst still continuing with Flohale 125 CFC free inhaler.
Possible systemic effects include Cushing's syndrome, Cushingoid features, adrenal suppression, growth retardation, decreased bone mineral density, cataract, glaucoma.

As with other inhalation therapy, paradoxical bronchospasm may occur. This should be treated immediately with a fast-acting inhaled bronchodilator. Flohale 125 CFC free should be discontinued immediately, the patient assessed, and if necessary alternative therapy instituted.

Overdose :
<u>*Acute*</u>:
Inhalation of the drug in doses in excess of those recommended may lead to temporary suppression of adrenal function. This does not necessitate emergency action being taken. In these patients treatment with fluticasone propionate by inhalation should be continued at a dose sufficient to control asthma adrenal function recovers in a few days and can be verified by measuring plasma cortisol.

<u>*Chronic*</u>:
Monitoring of adrenal reserve may be indicated. Treatment with inhaled fluticasone propionate should be continued at a dose sufficient to control asthma.

Storage:
Store below 30˚C. Do not freeze

Shelf life:
24 months.

Packing/Pack size:
Carton containing single unit of pressurized aluminium container of 120 MD

Date of Revision : March 2022

Registration Holder in Malaysia
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Cipla

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21096957

SPECIAL INSTRUCTION:
(FOR PACK INSERT)

1) THICKNESS OF FOLDED LEAFLET SHOULD BE 3 to 4 MM
2) PROOFLESS ARTWORK
3) Red colour will be given to Outline box of artwork
(This is only for illustration and DON'T PRINT)